

TITLE: URINE TESTING

S/N	ACTIVITIES	0	1/4	1/2	3/4	1
	Preparation					
1	Ensures that requirements are complete (1/2)					
2	Mentions that Albumin can be tested using albusix reagent strip, or Salicyl-sulphuric acid. Also hot test can be done using acetic acid. (1)					
	Procedure					
3	Washes and dries hands (1/2)					
	Observes for physical characteristics					
4	Observes the colour of the urine, deposits and perceives the odour. (1)					
5	Checks the amount using measuring jug (1/2)					
6	Measures the specific gravity using urinometer (1)					
7	Checks for reaction using lithmus paper. States that if blue lithmus paper turns red, urine is acidic while if red lithmus turns blue, urine is alkaline (1)					
	Specific test for albumin					
8	Removes one Albustix strip quickly and replaces cap immediately. (1/2)					
9	Dips the reagent strip into urine and shakes off excess moisture from strip. (1)					
10	Compares strip colour with colour chart and red up value (estimation of quantity of albumin). (1)					
11	Discards strip appropriately (1/2)					
	Finishing					
12	Documents procedure accordingly (1/2)					
13	Tidies the work area (1/2)					
14	Washes and dries hands (1/2)					
Total marks obtainable: 10marks		Total marks obtained =				

Question Station

- The kidney is composed of about 1million functional units called -----
 - Tubules
 - Nephrons
 - Medulla
 - Neurones
- Urine output of less than 100mls a day is termed -----
 - Anuria

- b. Oliguria
- c. Polyuria
- d. Enuresis

3. The following are reagents for urine testing **except**

- a. Benedict's solution
- b. Sulphuric acid
- c. Clinitest tablet
- d. Bicarbonate solution

4. Urine is clear and amber in colour due to the presence of -----

- a. creatinine
- b. oxalates
- c. ammonia
- d. urobilin

5. The specific gravity is between -----and -----.

- a. 1010 and 1020
- b. 1010 and 1030
- c. 1030 and 1040
- d. 1010 and 1015

6. The normal pH of urine ranges between ----- and -----.

- a. 4.5 to 8
- b. 6.0 to 7.5
- c. 3.5 to 7.5
- d. 1.0 to 8

7. A healthy adult passes ----- to ----- ml of urine per day.

- a. 1500 to 3000 ml
- b. 1000 to 3000 ml
- c. 1000 to 1500 ml
- d. 1500 to 4000 ml

8. The amount of urine produced and the specific gravity vary according to -----

- a. food intake and fluid intake
- b. intake and output chart
- c. health status of the individual
- d. the fluid intake and the amount of solute excreted

9. During sleep and muscular exercise urine production is -----

- a. increased
- b. decreased
- c. moderated
- d. exceeded

10. Ketonuria may indicate all **EXCEPT**
- Diabetes mellitus
 - Starvation
 - Disease of the urinary tract
 - Excessive vomiting

Marking Guide

- B
- A
- D
- D
- B
- A
- C
- D
- B
- C

TEPID SPONGING

S/N	ACTIVITIES	0	¼ mk	½ mk	1 mk
1	Explain procedure to the client and gain consent (½)				
2	Take the prepared trolley to the client bedside (¼)				
3	Maintain privacy (¼)				
4	Wash and dry hands (½)				
5	Undress the client leaving the pubic area covered with lower towel(½)				
6	Place the bath sheet underneath the client (½)				

7	Put cold water in bowl and add hot water until it is tepid ($\frac{1}{2}$)				
8	Check temperature of water with bath thermometer or back of elbow ($\frac{1}{2}$)				
9	Then pick up face flannel and sponge the face, making a stroke and leaving drops of water ($\frac{1}{2}$ mk)				
10	Wrings out the flannel and apply to the forehead (1mk)				
	Then pick one body flannel and sponge the chest, hands starting from the one away from you, then the trunk, leaving the drops of water on the skin ($\frac{1}{2}$ mk)				
12	Applies cold compresses at the axilla ($\frac{1}{2}$ mk)				
13	Picks another body flannel and sponge the legs starting from the one away from you ($\frac{1}{2}$ mk)				
14	Turns the clients back ($\frac{1}{4}$)				
15	Using the Same flannel sponge the back, drop the flannel ($\frac{1}{2}$)				
16	Finally use the third body flannel and sponge the pubic area ($\frac{1}{2}$ mk)				
17	Changing water when necessary ($\frac{1}{4}$)				
18	Observe the general condition for the client e.g check for rigors ($\frac{1}{2}$)				
19	Check the client body temperature ($\frac{1}{4}$)				
20	Mentions that he/she will repeat the sponging if necessary ($\frac{1}{2}$)				
21	Make client comfortable ($\frac{1}{4}$)				
22	Mentions that he/she will take the temperature 30 minutes later and chart the true fall in temperature in red ($\frac{1}{2}$)				
TOTAL MARKS:10MKS		SCORE OBTAINED			

Question Station

- All are factors that can affect the body's heat production except
 - Basal metabolic rate
 - Muscle activity/exercise
 - Inflammation/fever
 - Evaporation
- Heat is lost from the body through;
 - Radiation, conduction, convection and evaporation
 - Conduction, radiation, radiotherapy, evaporation
 - Convection, conduction, radiotherapy, evaporation
 - Evaporation, radiation, radiotherapy, convection

3. A person with hyperthermia is given which of the following treatment
 - a. Cold sponging
 - b. Tepid sponging
 - c. Warm sponging
 - d. Hot sponging
4. Hyperthermia is defined as temperature that is above _____ °C
 - a. 37°C
 - b. 39.5°C
 - c. 40°C
 - d. 42°C
5. Convert the following temperature from Celsius to Fahrenheit. 37.5°C
6. Convert the following temperature from Fahrenheit to Celsius. 102.8°F
7. _____ gives the most accurate measurement of body heat.
 - a. Oral temperature
 - b. Axillary temperature
 - c. Rectal temperature
 - d. Brachial temperature
8. The recommended method(s) for temperature measurement in children under 4 weeks comprises
 - a. Electronic thermometer used orally
 - b. Infrared tympanic thermometer
 - c. Electronic thermometer in axilla
 - d. Analogue thermometer in axilla
9. The recommended dose of paracetamol in a child aged 5 years is
 - a. 250mg every 4-6hours
 - b. 125mg every 4-6hours
 - c. 500mg every 4-6hours
 - d. 200mg every 4-6hours
10. The part of the brain responsible for heat regulation is called _____

MARKING GUIDE

1. D
2. A
3. D
4. C
5. 99.5°F
6. 39.3°C
7. C
8. C
9. A

Hypothalamus

EDUCATE A STUDENT ON THE PREVENTION OF PRESSURE INJURIES

S/N	ACTIVITIES	0mk	¼mk	½mk	¾mk	1mk
1	Greets and introduces self. (½ mk)					
2	Ask patient what she knows about pressure sores. (½ mk)					
3	Defining pressure sores as localized damage to the skin and/or underlying tissues that usually occur over a bony prominence as a result of long term pressure, shearing, friction and moisture. (1mk)					
4	Major causes of pressure ulcers include; pressure, moisture, friction and shearing. (1mk)					
5	Mentions areas that are affected by pressure sores include; back of the head, shoulder blades, the elbows, lateral aspects of the hip joints, the buttocks, heels, lateral aspect of the knees, thoracic and sacral regions. (1mk)					
	PREVENTION					
6	Prevention includes; check the skin for signs of developing pressure sores often. (½mk)					
7	Use pillows to position the patient and increase comfort. (½mk)					

8	Emphasize on regular position changes and frequent turning of the patient every 2hours. (½mk)					
9	Eliminate sheet wrinkles that could increase pressure. (½mk)					
10	Ensure patient's heel do not rest on the bed. (½mk)					
11	Avoid rigorous massage on the pressure areas(½mk)					
12	Implement active or passive range of motion exercises to redistribute pressure and circulation. (½mk)					
13	Ensure patient doesn't lie on a soiled linen. (½mk)					
14	Recommend diet that includes adequate calories, proteins, vitamins and fibre rich foods (½mk)					
15	Use of bed appliances such as air ring or special beddings to reduce pressure. ½mk					
16	Ask client if she understands your teaching and entertaining her questions. (½mk)					
17	Thank her for her cooperation. (½ mk)					
Total mark obtainable: 10marks] Total mark obtained:						

QUESTION STATION

- The use of wheelchair is one of the factors that predispose one to pressure ulcer. **True / False**
- Cardinal signs of pressure ulcer in order of occurrence include
 - Redness-pain-warmth-blisters
 - Warmth-pain-blisters-redness
 - Pain-warmth-redness-blisters
 - None of the above
- When pressure ulcer is at the stage of an open sunken hole which makes the body fat visible. The pressure ulcer is said to be at
 - Stage I
 - Stage II
 - Stage III
 - Stage IV
- Signs that indicate that a pressure ulcer is infected and as such requires immediate significant intervention include
 - Fever and a foul odor from the ulcer
 - Break in the continuity of the skin

- c. Pain
 - d. All of the above
5. Other name(s) for pressure ulcer include
- a. Pressure sore
 - b. Decubitus ulcer
 - c. Pressure injury
 - d. All of the above
6. Conditions that can predispose a patient to pressure ulcer include all **except**
- a. Incontinence
 - b. Malnutrition
 - c. Advance in age
7. Prolong standing Ways of preventing pressure ulcer includes all **except**
- a. Reducing moisture
 - b. Skin massaging
 - c. Adequate Nutrition
 - d. Positioning and repositioning

Two bed accessories used for the prevention of pressure ulcers are

8. _____ and

9. _____

The assessment tools for pressure ulcer include

10. _____ and _____

MARKING GUIDE

- 1. True
- 2. A
- 3. C
- 4. A
- 5. D
- 6. D
- 7. B
- 8. Water bed

9. Air ring.

Bed cradle

10. Norton scale

Waterlow scale

Braden scale

TAKING GENERAL, PERSONAL, FAMILY AND SOCIAL HEALTH HISTORY

S/N	ACTIVITIES	0	1/4	1/2
1	Greets client and introduces self. $\frac{1}{4}$			
2	Explains the procedure to the client. $\frac{1}{4}$			
3	Sits beside the client. $\frac{1}{4}$			
4	Asks client how his health and wellbeing has been. $\frac{1}{4}$			
5	Asks about frequency of visit to a physician and related complaints $\frac{1}{4}$			
6	Asks about knowledge of preventive health practices $\frac{1}{4}$			
7	Asks about daily patterns of food and fluid intake- food preferences, special diets, food restrictions, appetite, actual weight, weight gain or loss. $\frac{1}{2}$			
8	Ask about patterns of bowel and bladder eliminations. $\frac{1}{2}$			
9	Ask about patterns of exercise, activity, leisure and recreation, ability to perform activities of daily living. $\frac{1}{2}$			
10	Ask about patterns of sleep, rest and relaxation $\frac{1}{4}$			
11	Ask about sensory perceptual patterns, memory and decision making ability. $\frac{1}{4}$			
12	Ask about patterns of self-concept, worth, emotional pattern and body image. $\frac{1}{4}$			
13	Ask about patterns of role engagement and relationship $\frac{1}{4}$			
14	Asks about client's satisfaction and dissatisfaction with sexuality pattern, client's reproductive pattern, premenopausal and menopausal problems. $\frac{1}{2}$			
15	Asks about client's ability to manage stress, sources of support, effectiveness of the pattern in terms of stress tolerance. $\frac{1}{2}$			
16	Asks about patterns of values, beliefs, spiritual practices and goals that guide the client's choices or decisions. $\frac{1}{4}$			

17	Asks client about personal information such as: a. Age ¼ b. Home address ¼ c. Residential address ¼ d. Position in family ¼ e. Religion ¼ f. Cultural beliefs ¼ g. Drug and food reactions ½			
18	Asks about family history such as: a. Parental occupation ¼ b. Location of residence ¼ c. Family relationships ¼			
19	Asks about client's social history such as: a. Educational attainment ¼ b. Occupation ¼ c. Food preferences ¼ d. Alcohol and Cigarette intake ¼			
20	Looks at the client during history taking ¼			
21	Listens attentively to the client's response ¼			
22	Uses more probing questions to verify response ¼			
23	Documents information as given by the client ¼			
	Total marks obtainable: 10marks] Total mark obtained:			

QUESTION STATION

- An oral, written or computer based communication intended to convey information to others is called ____
 - Recording
 - Reporting
 - Documentation
 - History taking
- A chart or place where formal or legal document that provides evidence of a client's care are found is called----
 - Record
 - Report
 - File
 - Document
- All are qualities of a good documentation on client's data **except**
 - Timely and complete
 - Accurate and confidential
 - Specific to the client
 - None of the above
- All are involved in orientation of patient during admission **except**
 - Take patient round the ward

- b. Demonstrate and explain how to use bed side equipment
 - c. Document all information
 - d. Indicate visiting hours
5. During admission, patient's needs are identified through _____
- a. Interacting with the patient
 - b. History taking and doctor's diagnosis
 - c. Physical assessment and observation
 - d. History taking and observation
6. The first thing the nurse do on admission is
- a. Greets, receives patient cheerfully into the ward and identifies patient by name
 - b. Introduces self to the patient and identifies patient by name
 - c. Identifies patient by name and greets patient and offers seat
 - d. Offers seat to patient and introduces self
7. All are moments for head to toe physical examination **except**
- a. During admission
 - b. During antenatal booking
 - c. At the beginning of each shift
 - d. In an emergency situation
8. One out of the following is **not** necessary during inspection
- a. Appropriate exposure of the body area to be inspected
 - b. Attentive listening
 - c. Careful observation
 - d. Adequate lightening

Outline any four(4) functional health pattern in Gordon's typology

9. _____

10. _____

MARKING GUIDE

- 1. B
- 2. A
- 3. D
- 4. C
- 5. C
- 6. A

7. D

8. B

9.to 10. Health perception and health management

Nutrition and metabolism

Elimination

Activity and exercise

Cognition and perception

Sleep and rest

Self-perception and self-concept

Roles and relationship

Sexuality and reproduction

Coping and stress tolerance

Values and belief

(Any four is correct)

RIGHT LATERAL POSITION

S/N	Activities	0	1/4	1/2	3/4	1
	Preparation					
1	Greets client and introduces self. $\frac{1}{2}$					
2	Explains the procedure to the client and obtain consent. $\frac{1}{2}$					
	Procedure					
3	Screens client. $\frac{1}{4}$					
4	Washes and dries hands. $\frac{1}{4}$					
5	Dons gloves $\frac{1}{4}$					
6	Adjusts the head of the bed to a flat position $\frac{3}{4}$					
7	Locks the wheels on the bed $\frac{1}{2}$					
8	Moves the client closer to the right side of the bed $\frac{1}{2}$					
9	Stands on the side of the bed nearest to the client, places the client near arm across the chest, abducts the far shoulder slightly from the side of the body and externally rotates it 1 mark					
10	Places one hand on the client's far hip and the other on the far shoulder, instructs the assistant to do same, and together roll the client to the right lateral position. 1 mark					
11	Places one hand on the client's far hip and the other on the far shoulder, instructs the assistant to do same, and together roll the client to the right lateral position 1 mark					
12	Places pillows to support the head, neck and upper segment of the arm $\frac{1}{2}$					
13	Ensures top hip and knee are flexed $\frac{1}{2}$					
14	Ensure upper arm and shoulder are rotated internally and adducted with a pillow under the upper segment of the arm $\frac{3}{4}$					

15	Ensure upper thigh and leg are rotated internally and adducted with pillow under the leg and thigh 1mark					
	Finishing					
16	Tidies the work area ¹ / ₄					
17	Document procedure accordingly ¹ / ₂					
	Total marks obtainable: 10marks] Total marks obtained:					

QUESTION STATION

- In lateral position most of the body weight is borne by the following **except**;
 - The lateral aspect of the lower scapular
 - The lateral aspect of the ileum
 - The greater trochanter of the femur
 - The lateral aspect of the thoracic cage
- The following are true about the lateral position **except**;
 - It is good for sleeping and resting clients
 - The flexion of top hip and knee reduces lordosis
 - The greater the flexion of the lower hip and knee, the greater the stability and balance of the position
 - It helps to reduce

The following preventive actions are carried out in placing the client in the lateral position.
Match the likely problems they prevent against the preventive measures

S/N	Preventive measures	Likely problems they prevent
3.	Pillows under head and neck	a. Internal rotation and abduction of shoulder and subsequent limited function
4.	Pillows under upper arm	b. Internal rotation and abduction of the femur
5.	Flexion of the lower arm	c. Lateral flexion and fatigue of the sternocleidomastoid muscle
6.	Pillows under leg and thigh	d. Twisting of the spine
7.	Alignment of shoulder and hip	e. Impaired chest expansion
		f. External rotation of the leg

- What type of patient would benefit from an elevated head of the bed?
 - Patient with burns of the face and head
 - Patient with a broken femur
 - Patient who had a hemorrhoidectomy
 - Patient who had a lumbar puncture
- A patient is recovering from a mastectomy. Which answer is incorrect in regards to positioning?
 - Place the patient in semi fowler's position
 - Place the affected arm on a pillow

- c. Place the patient in a side-lying position on the unaffected side to promote lymphatic drainage
 - d. Every 2hours turn the patient on the prone position on the affected side
10. How would you position the patient during liver biopsy
- a. Left lateral position
 - b. Supine with right arm raised
 - c. Sim`s position
 - d. High Fowler`s position

MARKING GUIDE

- 1. C
- 2. D
- 3. C
- 4. A
- 5. E
- 6. B
- 7. D
- 8. A
- 9. C
- 10.B

EAR IRRIGATION

S/N	Activities	0	1/4	1/2	3/4	1
	Preparation					
1	Greet the client and introduces self. ¹ / ₂					
2	Explain the procedure to the client and obtain consent. ¹ / ₂					
	Procedure					
3	Screen the client. ¹ / ₄					
4	Washes and dries hands. ¹ / ₂					

5	Dons gloves. $\frac{1}{4}$					
6	Assist client to sit up or to lie with the head tilted towards the side of the left ear. $\frac{3}{4}$					
7	Instructs the client to support the receptacle under the ear to receive the irrigating applicator. 1mark					
8	Cleans the pinna and meatus of the auditory canal with saline soaked swab or cotton applicator. 1mark					
9	Fills the syringe with warm solution. $\frac{1}{4}$					
10	Straightens the auditory canal by pulling the pinna upwards and backwards. $\frac{3}{4}$					
11	Direct a steady slow, stream of solution against the roof of the auditory canal carefully using only sufficient force to remove secretions. 1mark					
12	Allow solution to flow out unimpeded. $\frac{1}{2}$					
13	Place a cotton ball loosely in the auditory meatus when the irrigation is completed. $\frac{1}{2}$					
14	Place client on the side of the irrigated ear. $\frac{1}{2}$					
	Finishing					
15	Observe and discard the irrigation fluid. $\frac{1}{2}$					
16	Tidies work area. $\frac{1}{4}$					
17	Removes gloves, washes and dries hands. $\frac{1}{2}$					
18	Documents the procedure noting the appearance of the drainage and the client's response. $\frac{1}{2}$					
	Total marks obtainable: 10mks] Total marks obtained:					

QUESTION STATION

- The nurse withholds ear irrigation when the patient has
 - Hearing defect
 - Impacted cerumen
 - Damaged cerumen
 - Patient with high fever
- Common side effects of ear irrigation include all except
 - Temporal dizziness
 - Ear canal discomfort or pain
 - Tinnitus, or ringing in the ear
 - Vitiligo
- After ear irrigation, what should the nurse tell the patient about the possible side effects?
 - Drink a lot of fluid and maintain bed rest
 - Side effect are typically short-lasting and go away within a day
 - Come to hospital immediately

- d. Use cold compress when symptoms begin to appear
- 4. The following are possible complications of ear irrigation the nurse should possibly watch out for
 - a. Ear infection
 - b. Perforated tympanic membrane
 - c. Vertigo
 - d. Damage to auditory nerve
- 5. Natural oil instillation into the ear is meant to
 - a. Trap bacteria
 - b. Soften impacted cerumen
 - c. Dislodge wax
 - d. Treat otitis media
- 6. The ear bone that transmit vibrations to the oval window of the cochlea are found in the
 - a. Inner ear
 - b. Outer ear
 - c. Middle ear
 - d. Eustachian tube
- 7. The instrument used to examine the ear is called
 - a. Eroscope
 - b. Otoscope
 - c. Oroscope
 - d. Aeroscope
- 8. Infection of the outer canal of the ear is known as
 - a. Otitis eterna
 - b. Otitis media
 - c. Otitis medius
 - d. Otitis externa
- 9. Which cranial nerve is responsible for the sense of hearing and balance
 - a. 4th cranial nerve
 - b. 6th cranial nerve
 - c. 7th cranial nerve
 - d. 8th cranial nerve
- 10. Between the bony and the membranous labyrinth within the internal ear is a layer of watery fluid called
 - a. Tympanic membrane
 - b. Perilymph
 - c. Endolymph
 - d. Oval window

Marking Guide

1. C
2. D
3. B
4. D
5. B
6. C
7. B
8. D
9. D
10. B

TRANSFER OF PATIENT FROM BED TO WHEELCHAIR

S/N	ACTIVITIES	0	$\frac{1}{4}$ mk	$\frac{1}{2}$ mk	$\frac{3}{4}$ mk
1	Greet client, explain procedure to client and obtain consent. $\frac{1}{2}$				
2	Wash hands, dry and put on gloves. $\frac{1}{4}$				
3	Get the gait belt and the wheelchair close to client's bed side. $\frac{1}{4}$				
4	Move the client close to the side of exit and place client in high fowler's position. $\frac{1}{2}$				
5	Wear the client his slippers or shoes. $\frac{1}{4}$				
6	Bring the bed down to its lowest and lock the wheels. $\frac{1}{4}$				
7	Place the chair with its side next to the side of the bed facing towards the foot of the bed. $\frac{1}{2}$				
8	Lock the wheels on the wheelchair and move the foot rest out of position. $\frac{1}{2}$				
9	Lower the side rails between the client and the chair if any. $\frac{1}{4}$				
10	Assist the client to a dangling position and wait for some moment to determine if client is lightheaded. $\frac{1}{4}$				
11	When client feels comfortable and steady, support the client with the gait belt by placing it around the client's waist line. $\frac{1}{2}$				
12	Leave enough slack to slip two hands in between the belt and the body. $\frac{1}{2}$				
13	Bend your knees and hips so that the trunk will be at the same level with the client's trunk. $\frac{1}{2}$				
14	Place feet in a wide stance with one foot forward. $\frac{1}{4}$				
15	Instruct client to place arm and hands on the waist or on the bed if strong enough. $\frac{1}{4}$				
16	Grasp the belt above on both sides of the client. $\frac{1}{4}$				

17	Help client to move buttocks towards the edge of the bed so that the feet touches the floor or close to the floor. $\frac{1}{2}$				
18	Instruct the client to rise slowly to a standing position. $\frac{1}{4}$				
19	Continue supporting the client by holding the waist belt and raising your body as the client rises. $\frac{1}{4}$				
20	Place your front knee against the client's opposite knee and move the body with a backward motion and steady the client when standing. $\frac{1}{2}$				
21	Help the client move his body gradually until the back is next to the chair seat. $\frac{1}{4}$				
22	Instruct client to sit down when the edge of the chair seat is felt against the back of the leg placing the hands on the arm of the chair. $\frac{1}{2}$				
23	Instruct client to gently lower his body to sit down while you hold the belt firmly until the client is seated. $\frac{1}{2}$				
24	Assess client to determine correct body alignment. $\frac{1}{4}$				
25	Help client raise legs and reposition the foot rest on the wheelchair and help the client securely place feet and legs on the rest. $\frac{3}{4}$				
26	Place a safe belt around the client's body if necessary. $\frac{1}{4}$				
27	Unlock the wheelchair before moving it. $\frac{1}{4}$				
	Total mark obtainable: 10marks] Total mark obtained:				

QUESTION STATION

- Assisting clients change position helps to ____
 - Improve blood circulation
 - Prevent pressure ulcers
 - Promote clients comfort
 - All of the above
- Mechanical lift are used when ____
 - Clients weight make it unsafe to move them
 - Clients are unable to stand on their own
 - Clients need to do some exercise
 - A and B
- All are good body mechanics **except**
 - Bend from hip and knees
 - Use strong thin muscle for lifting
 - Bend from back
 - Stand with feet wide apart enough to give a good base for working
- Wrist watches and rings are removed before moving and lifting a patient. **True / False**

5. Orthopnoeic position are used in all **except**
- In severe dyspnea
 - In cardiac conditions
 - During resting, eating or drinking
 - To promote breathing

Positions used for certain types of postural drainage are

6. _____
7. _____

Three major classifications of exercise include

8. _____
9. _____
10. _____

MARKING GUIDE

- D
- D
- C
- True
- C
- Knee chest position
- Trendelenberg position
- Aerobic exercise
- Anaerobic exercise
- Flexibility exercise

TITLE: OPHTHALMIC ADMINISTRATION

Candidate's No: _____

S/N	ACTIVITIES	0	1/4	1/2	1
1.	Explain procedure to obtain consent.1/2mk				
2.	Washes hand and put on gloves. 1/2mk				
3.	Position patient to sit back with neck slightly hyperextended or lie down.1/2mk				

4. Consult patient chart for drug,dose,date and time.1/2mk				
5. Confirm expiration date of drug.1/2mk				
6. Confirm which eye is being treated because different medication or doses may be ordered for each eye.1/2mk				
7. Remove any discharge by cleaning around the eye with moistened swab,with eye closed from inner to the outer canthus,using a fresh swab for each stroke.1/2mk				
8. To remove crusted secretions around the eye, moisten a swab ask patient to close eye and then place the swab over it for 2 minutes.remove the swab and then continue until secretions are soft enough to be removed without injuring the eye mucosa.1/2mk				
9. Have the patient to sit or lie supine.instruct to tilt his head back and toward the side of the affected eye.1/2mk				
10. Remove the dropper cap and avoid contaminating the dropper tip or bottle top.1/2mk				
11. Before instilling the eye drops, instruct the patient to look up and away .this position moves the cornea away from the lower lid and minimizes the risk of touching the cornea with the dropper tip if patient blinks.1mk				
12. Gently pull down the lower lid of the affected eye and instill the drops in the conjunctival sac.try to avoid placing .1mk				
13. Use a fresh clean tissue to collect / remove any excess solution from the eye.1/2mk				
14. After instillation ask patient to close his eye gently,and also ask patient to blink.1/2mk				
15. Make patient comfortable.1/2mk				
16. Return medication to storage area and keep safe.1/2mk				
17. Remove and discard gloves.1/2mk				
18. Perform hand hygiene.1/2mk				
19. Document the procedure.1/2mk				
TOTAL=10MARKS				

QUESTION STATION

- During ophthalmic administration, if an ointment and drop have been ordered the drops should be instilled first. TRUE/FALSE.
- _____ and _____ are risk associated with unsafe injection practices except.
 - Hiv/Aids and Tetanus.
 - Malaria and Abscess
 - Hepatitis B and Pain
 - Hepatitis B and Abscess.
- The route of choice when large amount of drug is to be administered is?

- a. Sublingual route
 - b. Intramuscular route
 - c. Intravenous route
 - d. Intrathecal route
4. The normal absorption time for drugs given via the oral route is?
- a. 5minutes
 - b. 15minutes
 - c. 20minutes
 - d. 30minutes
5. The use of methylated spirit to clean an area prior to intramuscular injection is primarily intended to
- a. Prevention of needle contamination
 - b. Remove dead epithelial cells and make injection less painful
 - c. Remove dirt and promote drug distribution
 - d. Promote vasodilation and drug absorption
6. Drugs that bind to beta-adrenoreceptors and prevent the binding of norepinephrine and epinephrine at their receptors are called
- a. Epinephrine blockers
 - b. Norepinephrine antagonists
 - c. Beta adrenoreceptors agonists
 - d. Beta adrenoreceptors antagonists
7. The following are nonselective beta blockers **except** _____
- a. Propranolol
 - b. Acebutolol
 - c. Timolol
 - d. Penbutalol
8. Calculate the IV flow rate for 250 mL of 0.5% destrose to be administered over 180 minutes. The infusion set has drop factor of 30

The two major forms of drug interactions to be considered while administering medication are

9. _____ and
10. _____

Marking Guide

- 1. TRUE
- 2. D
- 3. C
- 4. D

5. A

6. D

7. B

8. Formula = $\frac{\text{Vol in ml}}{\text{Time in minute}} \times \frac{\text{Drop factor}}{1}$

$$- \frac{250\text{ml}}{180} \times \underline{30} \quad \frac{\underline{250}}{180} \times \underline{30}$$
$$\underline{42\text{drops per minute}}$$

9. Drug-drug interaction

10. Drug-food; drug – alcohol

Drug-disease referred to as DDSIs